

Adrabetadex ARM One-Page Provider Economics Summary

Scenario: Selected scenario

Coding: N/A | Pass-through: N/A | Reimbursement: N/A

Site: N/A | Payer: N/A | 340B: N/A | Sourcing: N/A

WAC \$39,000 | Dosing Every 2 weeks | Annual administrations N/A | Annual gross WAC \$1,014,000

Planning assumptions: OPPS pass-through target Jan 1, 2027; permanent HCPCS target Apr 1, 2027; white-bagging payer-driven toggle Available; non-340B sensitivity/exception available

Annual Economics

Annual drug acquisition	N/A
Annual allowed drug reimbursement	N/A
Annual non-drug operating cost	N/A
Annual net contribution	N/A
Annual 340B acquisition advantage	N/A

Per-Dose Panel

Per-dose net margin	N/A
Drug acquisition per dose	N/A
Allowed reimbursement per dose	N/A
Non-drug operating cost per dose	N/A

Risk Flags

Cash exposure risk	None
Operational risk	None
Margin context	None

Flags are directional planning indicators. Verify payer-specific billing edits, authorization workflow, and payment-cycle performance before launch commitments.

Takeaways and Disclaimer

- Provider economics improve materially under 340B buy-and-bill relative to non-340B sensitivity assumptions.
- White-bagging is payer-driven and can reduce provider acquisition exposure while also reducing provider drug reimbursement.
- Planning assumptions for OPPS pass-through (Jan 1, 2027) and permanent HCPCS (Apr 1, 2027) should be verified against current payer and CMS guidance.

Interpretation:
At current assumptions, the modeled per-dose net margin is N/A and annual net contribution is N/A; provider results should be verified against payer policy, contract terms, coding status, and remittance timing before operational decisions.